

Adult Cardiac Arrest Checklist



On Arrival – BLS

- ☐ Team Leader identified?
- ☐ Is there enough room around the patient?
- ☐ Has the defib been attached?
- ☐ Defib pad placement optimal?
- ☐ Shockable rhythm?
- ☐ Is the airway being adequately maintained?
- ☐ Suction available?
- ☐ Is there sufficient oxygen?
- ☐ Is the BVM and reservoir bag fully inflated?
- ☐ Is the chest rise adequate?
- ☐ Are effective chest compressions ongoing?
 - 30:2
 - Rate 100 – 120 a minute
 - Depth 5 – 6 cm
- ☐ Are we switching clinician providing CPR regularly?
- ☐ If traumatic arrest has MERIT been considered?
- ☐ Has a second crew been requested?

After ALS Initiated

- ☐ Apply ETCO2 monitoring?
- ☐ Is there an ETCO2 waveform present and numerical value? If <1.5kpa – ask why?
- ☐ Is the ECG rhythm being assessed every 2 minutes?
- ☐ Is there bilateral air entry?
- ☐ Has vascular access been gained and patent? Consider external jugular vein access.
- ☐ Consider 4H's and 4T's and reverse where possible.
- ☐ Do drugs need to be given?
- ☐ Is a fluid challenge required?
- ☐ What is the blood sugar?
- ☐ Has a temperature been taken?
- ☐ Have all other reversible causes been considered?

Post ROSC

- ☐ Repeat primary survey
- ☐ What is the respiratory rate per minute?
 - <10 – ventilate 1 breath every 6 seconds
 - >10 – are we maintaining SPO2 to 94-98%
- ☐ Have we got a FULL set of observations?
 - Heart Rate
 - Blood Pressure
 - SPO2
 - Blood Glucose
 - 12 lead ECG
 - ETCO2
- ☐ What is the heart rate?
 - <60bpm and symptomatic has Atropine been given as per guidelines?
- ☐ What is the BP?
 - <90mmHg has a 250ml fluid bolus been administered?
- ☐ If agitated/combatative has CCP/Doctor assistance been requested?
- ☐ If 12 lead ECG shows a clear STEMI consider transport to a PPCI capable centre

Before Leaving Scene

- ☐ Will we be taking this patient to hospital?
- ☐ Has the team had a discussion regarding removal?
- ☐ Do we have the kit necessary for extrication?
- ☐ Enough oxygen on scene for removal?
- ☐ Family supported and communicated with?
- ☐ Hospital pre-alert completed.

4 H's – Hypoxia, Hypovolemia, Hyper/hypokalaemia, Hyper/hypothermia
4 T's – Toxins, Tamponade, Thrombosis, Tension pneumothorax

Paediatric Cardiac Arrest Checklist



On Arrival - BLS

- ☐ Someone is in charge – Team Leader?
- ☐ Is 360° access adequate? If not move patient
- ☐ Childs position optimised for airway patency?
- ☐ Is the airway being adequately maintained?
- ☐ Suction available?
- ☐ Is the oxygen connected and turned on?
- ☐ Is there sufficient oxygen?
- ☐ Paediatric BVM and correctly sized mask used?
- ☐ Is the BVM and reservoir bag fully inflated?
- ☐ Is the chest rise and fall adequate and gentle?
- ☐ Has the defib been attached?
- ☐ Defib pad placement optimal?
- ☐ Shockable rhythm?
- ☐ CPR at correct rate and effective?
 - 3:1 – immediately following birth
 - 15:2 – child up to puberty
 - 30:2 – over the age of puberty
- ☐ Has MERIT/CCP been considered?
- ☐ Has a second crew been requested?
- ☐ Has a member of staff been assigned to support and communicate with family?
- ☐ Where possible and appropriate family members should be allowed to witness resuscitation attempts.

After ALS Initiated

- ☐ Is airway patent and ventilations effective?
- ☐ Ventilations at correct rate? 12-24 breaths per minute according to normal values for age.
- ☐ Are we switching clinician providing CPR regularly?
- ☐ Apply ETCO2 monitoring?
- ☐ Is there an ETCO2 waveform present and numerical value? If <1.5kpa – ask why?
- ☐ Is the ECG rhythm being assessed every 2 minutes?
- ☐ Is there bilateral air entry?
- ☐ Has vascular access been gained and patent? Consider intraosseous access.
- ☐ Consider 4H's and 4T's and reverse where possible.
- ☐ Do drugs need to be given?
- ☐ Is a fluid challenge required?
- ☐ What is the blood sugar?
- ☐ Has a temperature been taken?
- ☐ Have all other reversible causes been considered?

Post ROSC Care

- ☐ Repeat primary survey
- ☐ Is airway patent and ventilations effective?
- ☐ Ventilations at correct rate? 12-24 breaths per minute according to normal values for age.
- ☐ Are we ensuring 'gentle' chest rise and fall?
- ☐ Observe for decreasing heart rate and fall in ETCO2 as this could indicate child may re-arrest
- ☐ If bradycardic ensure hypoxia is reversed
- ☐ Have we got a FULL set of observations?
 - Heart Rate
 - Blood Pressure
 - SPO2
 - Blood Glucose
 - Temperature
 - ETCO2
- ☐ Has a full history of events been obtained?
- ☐ Child may re-arrest – anticipate this and plan ahead

Leaving scene

- ☐ Has the team had a discussion regarding removal?
- ☐ Do we have the kit necessary for extrication?
- ☐ Do we have enough oxygen on scene for removal?
- ☐ Family supported and communicated with?
 - A parent/carer should travel with the patient where appropriate
 - Family members not travelling with the child know which hospital we are going to
- ☐ Hospital pre-alert completed, ensure a minimum 10-minute warning
- ☐ Clarify who is driving, ensure a steady but progressive drive
- ☐ Is the child appropriately secured to the stretcher and family wearing seatbelts?

A safeguarding referral MUST be made for all paediatric cardiac arrests along with completion of the Safeguarding Critical Incident Pack (SCIP)